

JOHNS HOPKINS MEDICINE – NEUROLOGY REFERRAL INTAKE
FAX RECEIVED: 04/12/2026 14:22
FROM: Green Valley Internal Medicine
FAX: (703) 555-8812

REFERRAL REQUEST (Cover Sheet)

Patient Name: Margaret L. Henson
DOB: 03/18/1948
Reason for Referral: memory issues / ?dementia / r/o Parkinsons?
Requested Service: Neurology (general?)
Priority: Routine (but family requesting sooner if possible)
Referring Provider: Daniel K. Brooks, MD
Insurance: Medicare + AARP supplemental

Additional notes scribbled: “pt getting worse past year, repeats self, husband concerned”

Page 1 of 11

PATIENT DEMOGRAPHICS

Printed: 04/10/2026 09:11

Name: Henson, Margaret Louise
MRN: 4492031
DOB: 03/18/1948 (Age 78)
Sex: F
Address: 1427 Maple Ridge Ct, Centreville VA 20120
Phone: (571) 555-2290
Marital Status: Married
Emergency Contact: Thomas Henson (spouse) (571) 555-2291

Primary Care: Daniel Brooks MD
Pharmacy: CVS #1832 Centreville

Allergies: NKDA (note: older record lists “sulfa – rash?” unclear)

Past Medical History (auto-populated):

HTN
Hyperlipidemia
Type 2 DM (diet controlled?)
Osteoarthritis
GERD
Seasonal allergies
“memory decline” (added 2025)

Surgical History:

Appendectomy (1965?)

Right knee arthroscopy (2009)

Social History:

Lives w/ husband

Retired school secretary

Denies tobacco

Occasional wine (1-2/week per pt; husband says none)

Family History:

Mother – “senile dementia” late 70s

Father – MI age 62

Page 2 of 11

REFERRING PROVIDER NOTE

Date: 04/08/2026

Chief Complaint: “forgetfulness worsening”

HPI:

78F w/ hx HTN, DM, HLD presents w/ husband for evaluation of progressive memory problems. Onset approx 2–3 yrs ago per spouse, initially mild (misplacing items, forgetting appointments), now more noticeable past 6–9 months. Repeats questions, difficulty recalling recent conversations. Still independent in ADLs but needs reminders for meds and bills. No wandering.

Denies headaches, vision changes, focal weakness. No seizures. Sleep okay. Appetite fair.

Pt herself states “just normal aging” and appears mildly defensive when questioned.

ROS:

Neuro: +memory issues, no dizziness

Psych: denies depression but affect somewhat flat

Cardiac: no CP

GI: +GERD

Exam:

Vitals stable

General: NAD

Neuro: alert, oriented x2 (missed date), recall 1/3 at 5 min, attention mildly impaired, no focal deficits, gait normal

Mini-Cog done last visit: abnormal

Assessment:

Cognitive decline, likely dementia vs other neurodegenerative process

Consider Alzheimer type vs vascular vs less likely Parkinsonian? (no tremor noted)

Plan:

Refer neurology for further evaluation

Labs reviewed (see attached)

MRI ordered previously (attached report)

Signature: D. Brooks MD

Page 3 of 11

PRIOR NOTE #1

Date: 11/14/2025

CC: follow-up DM

HPI:

Pt here for routine visit. Husband mentions memory "not as sharp." Pt denies issues. Occasional missed meds. Still driving short distances.

Exam:

Oriented x3

Recall 2/3

Gait steady

Assessment:

DM stable

Mild cognitive concerns

Plan:

Monitor

Consider cognitive testing if worsens

Page 4 of 11

PRIOR NOTE #2

Date: 05/02/2025

CC: annual wellness

HPI:

Pt reports doing well. No major complaints. Husband states she repeats stories. Pt laughs, says "he exaggerates."

Screening:
Mini-Cog borderline (clock drawing slightly off)

Assessment:
Possible mild cognitive impairment

Plan:
Labs
Reassess next visit

Page 5 of 11

PRIOR NOTE #3
Date: 01/18/2024

CC: cough

HPI:
Acute bronchitis. No mention of memory concerns.

Exam:
Neuro grossly intact
Oriented x3

Assessment:
Bronchitis
HTN

Plan:
Azithromycin

(note: copy-forward includes PMH identical to later notes)

Page 6 of 11

LAB RESULTS (various dates mixed)

CMP (04/2026):
Na 139
K 4.1
BUN 18
Cr 0.9
Glucose 122

CBC (04/2026):
WBC 6.2

Hgb 12.8
Plt 245

TSH (03/2026): 2.1

B12 (03/2026): 410

A1c (04/2026): 6.8

Lipid panel (04/2026):
LDL 102
HDL 52

Urinalysis (random, 02/2026): normal

Vitamin D (old, 2023): low (22)

Page 7 of 11

IMAGING

MRI BRAIN WITHOUT CONTRAST

Date: 03/21/2026

Findings:

Mild diffuse cerebral volume loss.

Prominence of ventricles and sulci consistent with age-related changes vs atrophy.

No acute infarct.

Scattered periventricular white matter hyperintensities, likely chronic microvascular ischemic changes.

Impression:

No acute intracranial abnormality.

Chronic changes as above.

(note: handwritten: “? hippocampal atrophy?”)

Page 8 of 11

MEDICATION LIST (active as of 04/2026)

Lisinopril 20 mg daily
Atorvastatin 20 mg nightly
Metformin 500 mg BID
Omeprazole 20 mg daily
Aspirin 81 mg daily

Calcium + Vitamin D
Loratadine PRN
Ibuprofen PRN knee pain
Multivitamin
Melatonin 5 mg nightly (started recently)

(old med still listed: ranitidine – likely discontinued)

Page 9 of 11

DISCHARGE SUMMARY (ER VISIT)

Facility: Fairfax Community Hospital

Date: 12/03/2025

Reason: confusion episode

HPI:

Pt brought by husband after brief episode of confusion, unable to recall where she was while shopping. Resolved by arrival.

Workup:

CT head negative

Labs unremarkable

Assessment:

Transient confusion, unclear etiology

Disposition:

Discharged home

Follow up PCP

(note: no recurrence documented in ER note)

Page 10 of 11

PROCEDURE NOTE

Mini-Mental State Exam

Date: 02/10/2026

Score: 22/30

Deficits in:

Recall (0/3)

Orientation (missed date and day)

Attention (serial 7s incomplete)

Interpretation: abnormal

(printed twice on page due to duplication error)

Mini-Mental State Exam

Date: 02/10/2026

Score: 22/30

Page 11 of 11

FAX TRAILER

Transmission complete 11 pages

Some pages slightly skewed / text cutoff on right margin

End of document